

رئيس
المجلس الطبي المتخصص

--

Name: سوزان انور مصطفى السيد National ID: 27503032101821	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

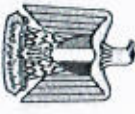
وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced MRI of the lumbar spine
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

DM5
asf

٢٠٢٥/٠٤ ٢٧٥.٣.٣٢١.١٨٢١
رَبِّهِ مَنَزَل
مَنْشِي
هَافِي مَحْطَقِي جَوهر السَّيِّد
الطَّاقَةُ سَارِيَّة حَتَّى ٢٠٣٢/٠٤/٢٨ مَرُوجِيَّة



سوزان
أنور محطقي السيد
١٦ ش. طابور التاجية
القي - الجزيرة

جَمَاهِيرِيَّةُ كُرْدِسْتَانِ
بِلادَةُ عَمَلِيَّةِ السَّيِّدِيَّةِ



٢٧٥.٣.٣٢١.١٨٢١
LC6224317

03/08/2026

MDT FORM

Name:	Souzan Anwar Moustafa /	MRN:	1260
Age:	51 Y	ID:	٤٧٥٢-٢٢١-١٨٤١
Menopausal Sta	post	Phone:	01066659144
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	hx of RT breast cancer >> mastectomy + implant	Reason Of MDT:	

Clinical Data:

BX:

ER: 8/8 PR: 2/8 HER2: +2 not ar KI67: 25%

Sonomammography:

Mets Workup:

ct cap >> tiny pul nodule + S1 mixed lytic / sclerotic osseous lesion

History:

It breast cancer was planned for NACT if bone scan -ve but bone scan showing active mets osseous lesion at LV3
for reMDT

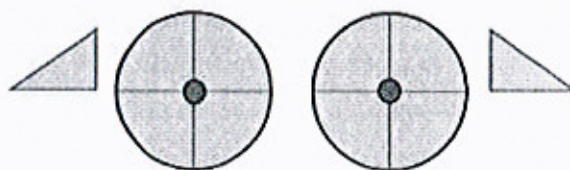
Recommendations:

for femara with zometa every 3 ms

MDT Outcome & Plan

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حنان عزت	د. احمد نجيب				

Name: سوزان انور مصطفى		
MR. : 1260		
Detailed Medical Report		
		السيد
تخية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient underwent a bone scan that demonstrated active metastatic disease involving the L3 vertebral body, and correlation with other radiological findings was requested. The patient has a history of invasive ductal carcinoma, grade II, hormone receptor–positive and HER2-negative. She received right-sided modified radical mastectomy in June 2019 for a pT3N1M0 tumor, followed by adjuvant chemotherapy consisting of three cycles of FEC and three cycles of docetaxel, then postoperative radiotherapy and five years of tamoxifen. In February 2026, a new left-breast mass was identified and biopsied as invasive ductal carcinoma with estrogen receptor 8/8, progesterone receptor 2/8, and HER2 2+. Breast MRI revealed multicentric lesions in the left breast at the 2- and 10-o’clock positions with pathological lymph nodes (T1, N2). A bone scan showed metastasis at L3. The multidisciplinary team discussed initiation of letrozole (Femara) and zoledronic acid (Zometa); the patient reported self-funded use of these agents for two months while awaiting formal approval, and a new approval request has been submitted. A CT of the chest, abdomen, and pelvis was performed, and the patient was cleared to continue letrozole while awaiting a repeat bone scan to assess metastatic breast cancer involvement of bone. The current plan is to maintain endocrine therapy with letrozole and monitor disease status with imaging.		
تحريرا في ,		
	التاريخ : 19-09-2026	الوقت :
	نرمين مصطفى عبد القادر بهي الدين	الطبيب المعالج :
يعتمد ,		

Name: Mrs. Suzan Anwar Mustafa Al-sayed
Date: 2025-11-27

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:
Clinical summary: Post right MRM & ALAND with silicone implant, complaining of left axillary lump of 1 month duration.

Findings:

Right breast.

- Almost entirely fatty breast parenchyma. (ACR a)
- Intact silicone implant with no sign of intra or extra-capsular rupture.
- Thickened skin.
- Ultrasound demonstrated intact silicone implant with no sign of intra or extra-capsular rupture
- No solid or cystic lesions or fluid collections.
- Thickened skin reaching 3.5 mm.
- Clear right axilla. No infra or supraclavicular lymph nodes.

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- Multiple irregular speculated high density mass lesions with surrounding distortion:
- Ultrasound demonstrated Multiple irregular spiculated heterogenous hypoechoic solid mass lesions with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study largest are seen at:
 - Index Lesion: 2 o'clock, 6 cm from nipple, measuring 13 x 16 mm.
 - 12 o'clock, measuring 10 x 5 mm.
 - 10 o'clock, 9 x 8 mm.
 - Multiple satellites are seen at the rest of the breast parenchyma measuring 4-7 mm each.
- 4 o'clock, non-specific intra-mammary lymph node measuring 4.4 x 6.5 mm with 2 mm cortical thickness.
- Normal skin/nipple/areola complex.
- Multiple level 1 and 2 pathological axillary LNs (more than three). No supraclavicular pathological axillary nodes.

Conclusion:-

Right breast (BIRADS 2): clear mastectomy bed and axilla with intact silicone implant.

Left breast (BIRADS 5): Multicentric pathological process with ipsilateral pathological axillary lymph nodes (level 1 & 2).

Recommendation:-

For CESM & biopsy and histopathological correlation.

Best Regards,

DR. Amal Hegazy, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with

Category & recommendations

recommendations:

Suspected Cancer Risk

BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
	A: mild suspicion.
	B: moderate suspicion.
	C: high suspicion.
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	



Patient Name: Suzan Anwar Moustafa Al Sayed	Date : 14/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 49 Year(s)
Medical ID: 1691/2019	Pat. ID: 11526

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM with implant reconstruction and left breast reduction mammoplasty

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the left breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions with intact implant
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

14/10/2024 12:16 PM



Pathology Report

Name: سوزان انور

Age: 43 years.

Clinical data: Right breast cancer.

Specimen type: Right MRM

Date: 22/6/2019

Lab. Number: 450

Hospital Number: 1691/2019

Gross Description:

Mastectomy specimen; 23x17x9 cm partly covered by skin ellipse; 22x16 cm with attached axilla 10x10 cm.

Sectioning revealed a retro-areolar ill-defined widely extending firm greyish white mass; 11x10x2 cm, located 1cm deep to areolar skin, with least deep margin 6cm. the mass was enclosing nodular area; 1x1 cm.

Axillary dissection revealed 18 nodules up to 1x1 cm.

Microscopic Examination:

Histopathological examination of sections prepared from the mass revealed invasive cores and tubules of moderately anaplastic ductal epithelial cells associated with 10% DCIS. Positive lymphatic tumor emboli and fair desmoplastic stroma.

The nipple, areola & resection margins were free of tumor tissue.

There were positive metastatic deposits in 1/18 axillary lymph nodes with focal extra-capsular extension.

Diagnosis:

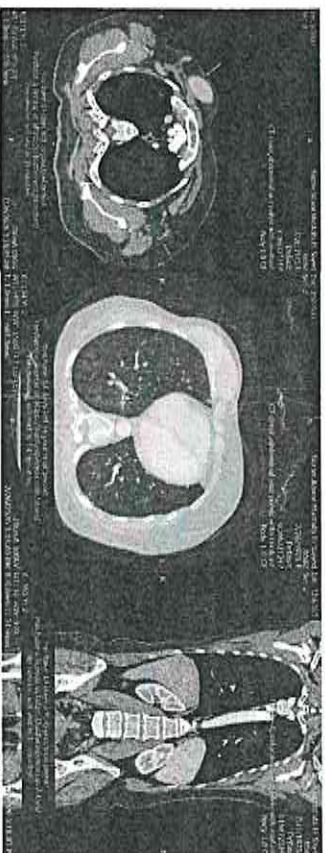
- Right MRM,
- Invasive duct carcinoma GII
- 10% DCIS
- Positive lymphatic tumor emboli.
- No perineural invasion.
- Free nipple, areola & resection margins.
- Positive 1/18 axillary lymph nodes with extra-capsular extension.
- Stage PT3 PN1.
- For IHC; (ER, PR, Her2 & Ki67)

Prof. Dr. Amal Haridy

Prof. Dr. Essam Ayad

Patient's Name:
Date:
Referred by:
Hospital:

Suzan Anwar Mustafa El-Sayed
2026-07-26
Prof.Dr. no doctor
Goustave Roussey International Egypt



**MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH
CONTRAST ADMINISTRATION REVEALED:-**

- Evident right mammoplasty with intact silicon stent seen inside and no evidences of either residual/recurrent or newly-developed neoplastic lesions seen in the breasts.
- Normal right axilla.
- **Benign -looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 11mm.**
- Hyperinflated both lungs.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- **Normal bones of the thoracic cage apart from spondylo-degenerative changes of the scanned spine.**
- Average sized liver,measuring about 14.4 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.7 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

Name : Suzan Anwar Mustafa

Date : 10-09-2026 03:53

Branch : Dokki

Medical No. : 459030

Ref.Physician Prof : Goustav Roussey Egypt

تعليق مستشفى : Ins.Company

NUCLEAR MEDICINE BONE SCAN REPORT

52-year-old female with breast carcinoma, status post left mastectomy, chemotherapy, and radiotherapy (2019), currently on hormonal therapy. Known bone involvement at L3, presenting with generalized bony aches.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Thoracolumbar scoliosis with rightward convexity is noted.
- A focal area of intense increased radiotracer uptake is identified at the L3 vertebral body, corresponding to the reported lesion.
- The remainder of the axial and appendicular skeleton shows symmetrical physiological tracer distribution without additional focal lesions.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

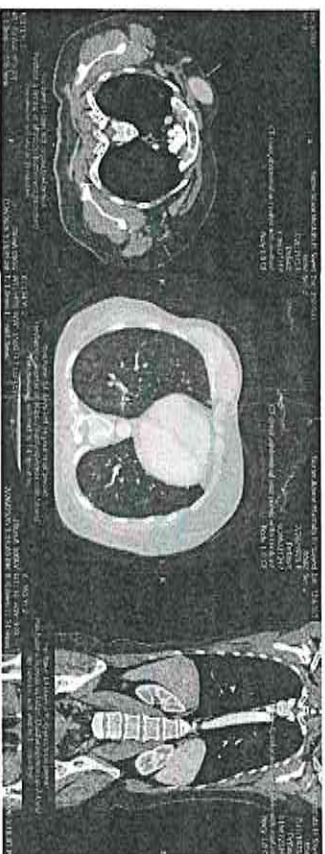
- Solitary focal osteoblastic lesion at L3, suspicious for secondary osseous metastatic involvement in the clinical context.
- No evidence of additional active osteoblastic metastases in axial or appendicular skeleton.
- Clinical and radiological follow-up as indicated.

Dr. Alaa Nagi

MD Nuclear Medicine

Patient's Name:
Date:
Referred by:
Hospital:

Suzan Anwar Mustafa El-Sayed
2026-07-26
Prof.Dr. no doctor
Goustave Roussey International Egypt



MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Evident right mammoplasty with intact silicon stent seen inside and no evidences of either residual/recurrent or newly-developed neoplastic lesions seen in the breasts.
- Normal right axilla.
- **Benign -looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 11mm.**
- Hyperinflated both lungs.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- **Normal bones of the thoracic cage apart from spondylo-degenerative changes of the scanned spine.**
- Average sized liver,measuring about 14.4 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.7 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

Name : Suzan Anwar Mustafa

Date : 10-09-2026 03:53

Branch : Dokki

Medical No. : 459030

Ref.Physician Prof : Goustav Roussey Egypt

تعليق مستشفى : Ins.Company

NUCLEAR MEDICINE BONE SCAN REPORT

52-year-old female with breast carcinoma, status post left mastectomy, chemotherapy, and radiotherapy (2019), currently on hormonal therapy. Known bone involvement at L3, presenting with generalized bony aches.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Thoracolumbar scoliosis with rightward convexity is noted.
- A focal area of intense increased radiotracer uptake is identified at the L3 vertebral body, corresponding to the reported lesion.
- The remainder of the axial and appendicular skeleton shows symmetrical physiological tracer distribution without additional focal lesions.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- Solitary focal osteoblastic lesion at L3, suspicious for secondary osseous metastatic involvement in the clinical context.
- No evidence of additional active osteoblastic metastases in axial or appendicular skeleton.
- Clinical and radiological follow-up as indicated.

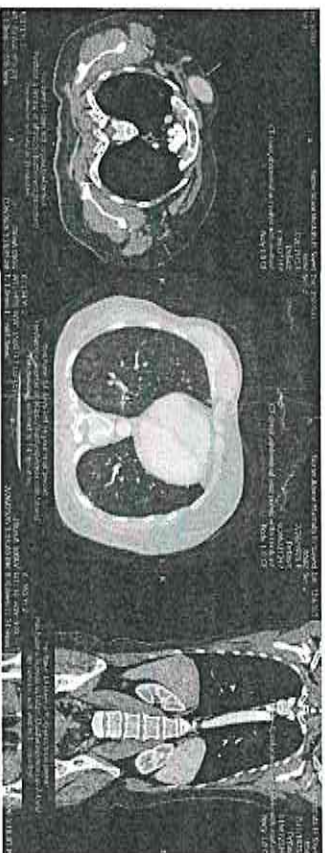
Dr. Alaa Nagi

MD Nuclear Medicine



Patient's Name:
Date:
Referred by:
Hospital:

Suzan Anwar Mustafa El-Sayed
2026-07-26
Prof.Dr. no doctor
Goustave Roussey International Egypt



**MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH
CONTRAST ADMINISTRATION REVEALED:-**

- Evident right mammoplasty with intact silicon stent seen inside and no evidences of either residual/recurrent or newly-developed neoplastic lesions seen in the breasts.
- Normal right axilla.
- **Benign -looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 11mm.**
- Hyperinflated both lungs.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- **Normal bones of the thoracic cage apart from spondylo-degenerative changes of the scanned spine.**
- Average sized liver,measuring about 14.4 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.7 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.



SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Suzan Anwar Mustafa

Date : 10-09-2026 03:53

Branch : Dokki

Medical No. : 459030

Ref.Physician Prof : Goustav Roussey Egypt

تعاقب مستشفيات : Ins.Company

NUCLEAR MEDICINE BONE SCAN REPORT

52-year-old female with breast carcinoma, status post left mastectomy, chemotherapy, and radiotherapy (2019), currently on hormonal therapy. Known bone involvement at L3, presenting with generalized bony aches.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Thoracolumbar scoliosis with rightward convexity is noted.
- A focal area of intense increased radiotracer uptake is identified at the L3 vertebral body, corresponding to the reported lesion.
- The remainder of the axial and appendicular skeleton shows symmetrical physiological tracer distribution without additional focal lesions.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- Solitary focal osteoblastic lesion at L3, suspicious for secondary osseous metastatic involvement in the clinical context.
- No evidence of additional active osteoblastic metastases in axial or appendicular skeleton.
- Clinical and radiological follow-up as indicated.

Dr. Alaa Nagi

MD Nuclear Medicine

N I L E D I A G N O S T I C C E N T E R